# PM-01 Pediatric Anaphylaxis / Allergic Reaction

**CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 26, 2026**

**Epinephrine is first-line treatment. For suspected anaphylaxis, give IM epinephrine immediately. Do not delay for IV/IO access, antihistamine, corticosteroid, albuterol, monitoring, or transport preparation.**

## Recognition and field flow

| Step | Approved action |
|---|---|
| **1. Assess** | Assess airway, breathing, circulation, skin/mucosa, GI and cardiovascular symptoms, perfusion, onset, and likely trigger. Remove the trigger when safe. |
| **2. Anaphylaxis?** | Treat when there is airway/breathing compromise, hypotension/poor perfusion, or acute involvement of 2 or more body systems after a likely allergen. Rash may be absent. |
| **3. IM epinephrine now** | Epinephrine 1 mg/mL: 0.01 mg/kg IM in the mid-outer thigh; maximum 0.3 mg prepubertal child or 0.5 mg adolescent. Repeat every 5 minutes while significant symptoms persist. |
| **4. Support ABCs** | Position supine when tolerated; oxygen, suction, BVM, and early airway planning as indicated. Give albuterol only for persistent lower-airway bronchospasm after epinephrine. |
| **5. Shock/refractory** | For hypotension or poor perfusion, give normal saline 20 mL/kg IV/IO and reassess after every bolus. Repeat IM epinephrine. Contact Medical Control early for refractory symptoms or epinephrine infusion. |
| **6. Reassess/transport** | Continuous ECG, SpO2, BP, respiratory, mental-status, and perfusion reassessment. Transport every child treated with epinephrine or with airway, respiratory, circulatory, progressive, or multisystem symptoms. |

## Provider-level actions

| Level | Approved actions |
|---|---|
| **EMT** | Remove the trigger when safe; support airway and ventilation; give authorized IM epinephrine for suspected anaphylaxis; position safely; monitor when available; and expedite transport. |
| **AEMT** | All EMT care plus rapid IV/IO access without delaying epinephrine, normal saline for anaphylactic shock with reassessment after each bolus, albuterol for bronchospasm, and authorized repeat IM epinephrine. |
| **Paramedic** | All prior care plus advanced airway support, Paramedic-only diphenhydramine for isolated or persistent cutaneous symptoms after stabilization, methylprednisolone as an adjunct, and Medical-Control-directed epinephrine infusion for refractory anaphylaxis after repeated IM doses. |

## Immediate safeguards

• Do not allow a hypotensive or poorly perfused child to stand or walk. A falling blood pressure may be late; poor perfusion or respiratory compromise warrants action before hypotension develops.

• Do not use intranasal epinephrine as the EMS anaphylaxis treatment pathway. Do not give a routine IV/IO epinephrine bolus for anaphylaxis.

• Albuterol treats lower-airway bronchospasm only. It does not reverse upper-airway edema, hypotension, or shock.

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**MEDICATIONS, REFRACTORY CARE, AND HANDOFF**

## Medication and fluid pathway

| Therapy | Dose and role |
|---|---|
| **Epinephrine 1 mg/mL** | 0.01 mg/kg IM in the mid-outer thigh; maximum 0.3 mg prepubertal child or 0.5 mg adolescent; repeat every 5 minutes as needed. |
| **Albuterol** | Under 20 kg: 2.5 mg nebulized. 20 kg or greater: 5 mg nebulized. Use only for lower-airway bronchospasm after epinephrine. |
| **Normal saline** | 20 mL/kg IV/IO for anaphylactic shock. Reassess perfusion, lungs, work of breathing, and response after every bolus. |
| **Diphenhydramine** | Paramedic: 1 mg/kg PO/IV/IO/IM; maximum 50 mg. Adjunct only for isolated or persistent cutaneous symptoms after stabilization. |
| **Methylprednisolone** | 2 mg/kg IV; maximum 125 mg. Adjunct only; delayed benefit. Do not delay or replace epinephrine. |
| **Epinephrine infusion** | Medical-Control-directed only for refractory anaphylaxis after repeated IM epinephrine. Use a labeled infusion and continuous monitoring; do not give a routine IV/IO epinephrine bolus. |

## Airway, refractory symptoms, and shock

• For progressive facial, tongue, or airway swelling, request airway resources early and use AR-05 Pediatric Airway and AR-06 Pediatric Failed Airway. Avoid repeated or traumatic airway attempts.

• If significant symptoms persist after repeat IM epinephrine, continue IM dosing every 5 minutes as indicated, provide airway and fluid support, contact Medical Control, and prepare a monitored epinephrine infusion only with explicit direction.

• Do not administer oral medication to a child with decreased mental status, vomiting, inability to swallow, or an unprotected airway. Avoid diphenhydramine in hypotension, poor perfusion, or decreased mental status.

## Transport, documentation, and linked pathways

| Topic | Required action |
|---|---|
| **Transport** | Every child treated with epinephrine, or with airway, respiratory, cardiovascular, progressive, or multisystem symptoms, requires ED evaluation and continuous reassessment for recurrence. |
| **Handoff** | Document trigger/exposure, onset, serial findings and vital signs, weight/length system, epinephrine concentration/dose/route/site/time, response, airway support, fluids, adjuncts, consultation, and destination notification. |
| **Links** | AR-05 Pediatric Airway; AR-06 Pediatric Failed Airway; AR-07 Pediatric Respiratory Distress; PM-03 Pediatric Hypotension / Shock; current Claiborne Medication Reference. |

**Critical reminder: Antihistamines and corticosteroids are adjuncts. They never replace epinephrine and must not delay its initial or repeat administration.**

## Clinical pearls

• The mid-outer thigh is the preferred IM epinephrine site for reliable absorption. If concern is high, give IM epinephrine rather than waiting for every diagnostic criterion.

• This direct source PDF replaces the prior imported sheet and matches Claiborne-approved PM-01 app content reconciled October 26, 2026.